CLINICAL TRIAL FAILURES · DISCONTINUATION RATE · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

EZH2: 2 molecules stopped early, all small molecules

3 stopped trials, but 2 molecules — all small molecules: Tazemetostat and Valemetostat. 1 of the stops were the trial's own verdict and 2 followed a decision taken elsewhere. Against 5.2% for all oncology trials with a resolved drug target and 4.5% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

21.4%
3 of 14 closed trials (95% CI 7.6%–47.6%)

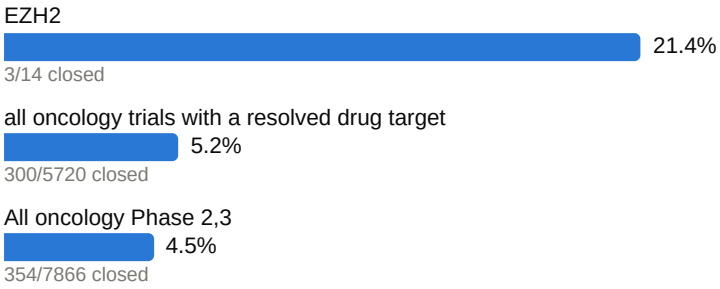
1 / 1 / 0
efficacy / safety / both (adds to 3)

28.6%
of 49 trials have closed · 35 still open or unresolved

0.0%
stopped for a biological reason within 3 years of starting (95% CI 0.0%–0.0%; 23 still at risk; 1.5% area-wide)

— **Completed and missed the primary endpoint.** None of the 9 completed trials in this segment posted a primary result on ClinicalTrials.gov that can be read, so nothing can be said here either way. 4 of the 8 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 74 distinct experimental drugs across 49 trials (small molecule 49, antibody 13, protein 4, unknown 3). Most tested: TAZEMETOSTAT (26), ZEPRUMETOSTAT (10), VALEMETOSTAT (7), RITUXIMAB (5), PREDNISONE (5). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 3 stops came from 3 sponsor-asset programmes across 3 sponsors; the largest (Prisma Health-Upstate) contributed 1; removing that programme's trials from both sides leaves 15.4%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. One further closed trial here was terminated with no cause recorded in the registry; if it were biological the rate would be 28.6%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2018–2020	2	6	33.3%	9.7%–70.0%

Where the stops sit

Benefit–risk: 1, Efficacy / futility: 1, Safety: 1.
Sponsors with most stops: Prisma Health-Upstate (1), Epizyme, Inc. (1), Memorial Sloan Kettering Cancer Center (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT05023655	2	Prisma Health-Upstate	TAZEMETOSTAT	2026-03	Benefit–risk	Ipsen has discontinued all active tazemetostat clinical trials and expanded access programs due to the emergence of a clinically unfavorable benefit risk profile for tazemetostat in combination with lenalidomide and rituximab.

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04179864	2	Epizyme, Inc.	TAZEMETOSTAT, ABIRATERONE, PREDNISONE	2024-11	Efficacy	The study was terminated early as Sponsor decided to discontinue the development of tazemetostat in metastatic castration resistant prostate cancer and the primary endpoint was not met for this study. There were no safety concerns.
NCT03879798	2	Memorial Sloan Kettering Cancer Center	VALEMETOSTAT, IRINOTECAN	2023-05	Safety	Due to DLTs

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 28.6% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. €99, delivered immediately: **clinicaltrialfailures.com/asset-check**

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.